

Cataract Option Brief

A plain comparison of Abuelita's options — prepared with love by Dana

This is not medical advice.

I am not Abuelita's doctor. The medical decision belongs to her ophthalmologist in Guadalajara and to you. What follows is a plain comparison of the options in front of us and what each one asks of the family. Read it, keep it, and bring the questions to Abuelita's ophthalmologist.

What we are looking at

Abuelita's cataracts have been getting worse. She has said her reading has become harder and the light at night bothers her more than it used to. Cataract surgery is one of the most common procedures done in the world, and for most patients recovery is short. For a woman of 80 with the health picture Abuelita has, the surgery is still routine, but the presence and support she needs during recovery is real and must be planned honestly.

Option A — Surgery in Guadalajara

The straightforward path. Abuelita's own ophthalmologist, the hospital the family already knows, and her own bed to recover in. Tia Lupe is on the ground and can be present for the pre-operative appointments. Cost in local currency is meaningfully lower than a cross-border version. Typical recovery timeline is a few days of close care with eye drops on a schedule, followed by follow-up appointments across four to six weeks. The first four days after surgery are the highest support days. Someone trusted needs to be at Abuelita's side those four days to help with the drops, to make sure she does not rub the eye, and to keep her routine steady.

What this asks of the family. Someone present for the first four days minimum. Ideally two family members overlapping so nobody is stretched. Tia Lupe can carry the pre-operative appointments. If the surgery is scheduled inside the January 25 through February 1, 2027 window, I can be there with you the whole time.

Option B — Surgery cross-border in the United States

The harder path. Abuelita would need to travel to the United States before the surgery and stay through recovery. She does not have US health insurance, so the surgery cost is roughly five to eight times higher than the Guadalajara version, and the surgeon would not be the ophthalmologist she has been seeing. Recovery would still ask for the same four days of close care after surgery, but away from her own bed and her own routine. The one reason this path would make sense is if there is a specific medical reason Abuelita's ophthalmologist recommends it. Absent that reason, this path is harder on Abuelita and harder on the family.

What this asks of the family. Travel logistics for Abuelita before and after surgery. Somebody hosting her for at least two to three weeks. Higher out of pocket cost. And a conversation with Abuelita's ophthalmologist about whether there is a clear reason to do this.

Comparison at a glance

Question	Option A — Guadalajara	Option B — Cross-border
Cost, relative	Lower	Meaningfully higher
Surgeon	Abuelita's own ophthalmologist	New surgeon, no history
Travel before surgery	None for Abuelita	Full international travel
Recovery location	Abuelita's own bed	Away from home
First 4 days of care	Family present in Guadalajara	Family present wherever surgery is done
Follow-up appointments	4 to 6 weeks locally	Complicated across borders
Insurance path	Existing local coverage	No US insurance, full out of pocket
Family logistics	Fly in for the window	Host for 2 to 3 weeks

Family presence, honest read

If the surgery is scheduled between January 25 and February 1, 2027, Dana can be in Guadalajara that full week. Val and Marco are planning to travel in the same window. Papa can fly in from San Antonio, Mama can fly with him or drive with him if he prefers. Tia Lupe is already in Guadalajara and can carry the pre-operative week. That is the best window for the family to be present without straining anybody's work or life. If the surgery lands outside that window, we regroup and look at March or April 2027.

What I could not verify

I could not verify the exact wait time for the surgery appointment at Abuelita's ophthalmologist. Tia Lupe can confirm this in person at the clinic and that is the trusted source. I could not verify the exact out of pocket cost of the cross-border version because the US surgery cost varies widely by hospital and by state. I could not verify whether Abuelita's ophthalmologist has any reason to recommend one option over the other, and this is the single most important question to ask him. I could not verify Abuelita's own preference between the two options, and this is the second most important question, and it belongs to Abuelita.

Questions for Abuelita's ophthalmologist

What is the wait time for a surgery appointment. Is there any medical reason to recommend one path over the other. What is the recovery protocol he uses. Is there anything about Abuelita's health picture at 80 that changes the standard recovery expectation. What does he need from the family for the follow-up appointments.

The decision belongs to Abuelita first

Before we compare costs and logistics, we ask Abuelita what she wants. What she worries about. Whether she wants to travel or stay home. Her answer comes first. This paper is here to help us think, not to decide for her.

With love, Dana.

This brief is for Mama, Papa, and Abuelita. It stays inside the family. It is not medical advice. The medical call belongs to Abuelita's ophthalmologist.